Field Visit Report

UNICEF - Management Accompaniment (MA) Project

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Abbreviations

Abbreviation	Meaning
AMC	Average Monthly Consumption
ANC	Antenatal Care
BPHS	Basic Package of Health Services
CHS	Community Health Supervisor
CHW	Community Health Worker
DHIS-2	District Health Information Software 2
EDM	Essential Drug Management
EPHS	Essential Package of Health Services
FP	Family Planning
GMP	Growth Monitoring and Promotion
HER	Health in Emergency Response
HFs	Health Facilities
HMIS	Health Management Information System
IYCF	Infant and Young Child Feeding
LHC	Local Health Committee
MA	Management Accompaniment
MSS	Minimum Service Standards
NSTG	National Standard Treatment Guidelines
P4P	Pay-for-Performance
PIP	Performance Improvement Plan
PNC	Postnatal Care
PPE	Personal Protective Equipment
QQM	Quantified Quality Matrix
SP	Service Provider
TPM	Third Party Monitoring
UNICEF	United Nations Children's Fund
WHO	World Health Organization

Executive Summary:

Under the Management Accompaniment (MA) Project, the MA officers of MgtWel and HPRO consortium successfully conducted field visits across 20 provinces in November 2025. These visits aimed to enhance provincial Service Providers' (SPs) technical and managerial capacity while developing Performance Improvement Plans (PIPs) using a new structured approach. During the visits, the MA team presented Provincial Data Packs, explained the updated scoring methodology, and identified gaps in performance across P4P indicators, HMIS verification, Quantified Quality Matrix (QQM) domains—including structural quality, content-of-care quality, and outcome quality—and Minimum Service Standards (MSS).

The team also provided on-the-job coaching and led group discussions to strengthen technical knowledge in clinical management, Essential Drug Management (EDM), hygiene, and service delivery processes. The PIPs were developed in a structured, stepwise approach, addressed gaps and supported sustainable improvement.

Key gaps included:

Low knowledge of SP staff on PIP development with new approach

Improper patient registration affecting HMIS verification

Incorrect diagnosis and treatment, including antibiotic overuse (>30%)

Low knowledge of pharmacists on AMC/EDM

Inadequate hygiene and infection prevention measures

Actions taken:

- Training of technical and managerial staff on the new PIP approach
- Group work exercises and presentations on PIP development
- Technical leadership by MA Officers during PIP preparation

Strategic Recommendations:

- Extend MA support to all 20 provinces with technical guidance for PIP development
- Construct health facility buildings where rental spaces are currently used
- Conduct comprehensive staff training on all performance attributes, especially content of care
- Motivate Welfare Organizations (INGOs & NGOs) to construct buildings for HFs where the HFs are currently exists in rental houses.
- The MA project should conduct more training on all performance attributs especially on the Content of Carc, to the HF staff of SPs in 34 provinces
- Review the current structure of the HF staff and increase the number of HF staff in some HF where required or due to the load of work on HF staff.
- Recruit additional Midwives and Medical Doctors by SPs in each province to replace those who are going on leave and avoid HF with no technical staff.

The field visits demonstrated the importance of combining supervision, technical mentoring, and structured planning to improve health service quality across Afghanistan.

Background:

The Health in Emergency Response (HER) program was designed to ensure continuity and quality of essential health services across Afghanistan through the implementation of the Basic Package of Health Services (BPHS) and Essential Package of Health Services (EPHS). Within this framework, the Management Accompaniment (MA) Project serves as a core mechanism to strengthen Service Provider (SP) performance by providing structured technical support, managerial mentoring, and data-informed decision-making at provincial and facility levels.

As part of Rounds 2 and 3 of the MA initiative under the HER 2 framework, MA Officers of MgtWell and HPRO consortium conducted provincial field engagements across 20 provinces to reinforce system functionality, accountability, and quality improvement processes. These visits were grounded in the recognition that persistent challenges—such as weak data use, inconsistent application of quality standards, and limited technical capacity—require hands-on accompaniment rather than remote supervision alone.

This phase of MA engagement marked a strategic shift from advisory support toward embedded technical leadership, emphasizing practical problem-solving, structured planning, and ownership by provincial SP teams. The introduction of a revised Performance Improvement Plan (PIP) methodology aimed to standardize improvement planning, strengthen alignment with HMIS and DHIS-2 findings, and promote evidence-based prioritization of actions.

The field visits also supported alignment with HER program objectives by strengthening linkages between performance data (HMIS, DHIS-2), quality improvement frameworks (QQM and MSS), and operational planning at provincial level.

By combining analytical review with direct engagement of SP leadership and frontline staff, the MA initiative sought to institutionalize a culture of continuous quality improvement, adaptive management, and accountability across BPHS and EPHS service delivery platforms.

Objectives:

The specific objectives for the provincial visits in this round of field visits and capacity building were:

1. Prepare Provincial Data Packs:

- Analyze the data and prepare the Provincial Data Packs.
- Identify gaps in indicators of the relevant attributes.
- Explain the updated scoring methodology.

2. Technical Leadership in the Development of Performance Improvement Plans (PIPs):

- Review previously developed PIPs at the provincial level.
- Explain the development of PIPs using the new approach.
- Technically lead the development of new PIPs, including the worksheet newly added by UNICEF for PIP development.

3. Group Discussion:

- Observe operational status, service delivery quality, and readiness across BPHS and EPHS facilities.
- Verify compliance with the Quantified Quality Checklist (QQC) and national standards.
- Provide a brief step-by-step explanation of usual disease management (History, Physical Exam, Lab Exam, Diagnosis, Treatment, and Consultation) by the MA Medical Specialists to improve diagnostic and treatment accuracy in all health facilities.
- Review Essential Drug Management and Tracer Drugs, including MAC (Monthly Average Consumption) calculations.

4. Deliver PIP Development Training with New Approach:

- Conduct PIP Development Training using the new approach for the Technical and Managerial Staff of SPs at the provincial level.
- Group team leaders present and explain the outcomes of the working groups to the participants of the PIP Development Team.

5. Strengthen Provincial Coordination and Communication:

- Establish direct engagement with provincial SP teams to bridge gaps between field-level and management practices.
- Share summaries of TPM scores in Data Packs and highlight identified gaps to initiate joint problem-solving.

6. Support Ethical and Behavioral Capacity Among Technical and Managerial Staff of SPs:

- Provide coaching on patient confidentiality, respectful communication, and professional conduct.
- Address staff concerns or dissatisfaction with TPM methodology through explanation and dialogue.

7. Inform HER 2 Strategy Development:

- Collect insights on staff capacity, infrastructure gaps, and service quality challenges.
- Document localized innovations, bottlenecks, and peer-learning examples for future scale-up.

Geographic Coverage:

During this round of field visits, the MA team covered twenty provinces across Afghanistan, ensuring representation from all major regions:

- Northeast: Badakhshan, Baghlan, Kunduz
- North: Saripul, Samangan
- East: Kunar, Laghman, Nuristan
- Southeast: Logar, Paktia, Paktika
- Central: Logar, Wardak, Kapisa, Panjshir
- Central Highlands: Daikundi
- West: Badghis, Farah
- South: Nimroz, Uruzgan, Zabul

These provinces were strategically included in the MA contract with MgtWell/HPRO, which provides comprehensive oversight of BPHS and EPHS service delivery and performance across diverse geographic, demographic, and operational contexts.

Rationale:

The field visits conducted by the MA teams across 20 provinces were designed to enhance implementation oversight, strengthen health facility performance, and translate Third Party Monitoring (TPM) findings into actionable improvements. These visits provided an opportunity to assess service delivery, identify performance gaps, and support Service Providers (SPs) in aligning with national quality standards and Minimum Service Standards (MSS).

As Afghanistan transitions from the HER 2 to the HER 3.0 framework, it is critical to reinforce SP capacities at the provincial level through direct engagement, technical mentorship, and evidence-based decision-making. The visits aimed to ensure that provincial teams not only understand the identified gaps but are equipped with practical strategies, tools, and guidance to improve service quality, enhance patient outcomes, and sustain health system performance.

Furthermore, the field visits facilitated knowledge sharing, strengthened coordination between SPs and provincial health authorities, and promoted consistency in the implementation of BPHS and EPHS services across diverse geographic and operational contexts.

Methodology:

The Management Accompaniment (MA) field visits were conducted using a structured, data-driven, and participatory methodology designed to identify performance gaps, analyze root causes, and technical lead the development of actionable Performance Improvement Plans (PIPs).

Data Sources:

Multiple data sources were triangulated to ensure accuracy and validity of findings:

- HMIS Data: Reviewed at provincial and facility levels to verify data accuracy, completeness, and consistency with reported performance indicators.
- DHIS-2 Data: Health Information Software - 2
- Provincial Data Packs: Developed and presented by the MA team to summarize provincial performance trends, scoring results, and identify gaps.

Approach to PIP Development and MA Support:

The MA team applied a new structured PIP development approach, implemented in five phases, emphasizing:

- Evidence-based gap identification using TPM and HMIS data
- Root cause analysis through facilitated group discussions
- Prioritization of feasible and high-impact actions
- Clear assignment of responsibilities and timelines
- Continuous technical mentoring and follow-up

MA officers provided hands-on technical leadership throughout the process, ensuring that PIPs were realistic, context-specific, and aligned with national standards and HER objectives.

Major Findings:

The major findings of the MA field visits to the 20 provinces of Afghanistan are challenges in HMIS verification, structural quality, content-of-care, and service outcomes. The 2025 field review of the Health in Emergency Response (HER) BPHS/EPHS program across 20 provinces of Afghanistan reveals persistent gaps in service delivery, systemic challenges, and opportunities for strengthening maternal and child health outcomes. The findings highlight variations in performance across provinces, reflecting differences in infrastructure, staffing, referral systems, and community engagement.

Badakhshan, Badghis, Farah, Sar-e Pul, and Samangan demonstrate widespread challenges in achieving P4P targets, including ANC, PNC, TT2+, GMP, IYCF, Penta-3, institutional delivery, and family planning. Weak internal and external referral systems, inadequate follow-up of defaulters, high dropout rates, and limited community awareness constrain service utilization. Cultural barriers and insufficient female health workers further reduce access to reproductive health services. Additionally, the presence of parallel vertical projects within catchment areas creates overlap and reporting challenges, contributing to incomplete HMIS documentation and reduced accuracy in performance verification.

Nimroz, Urozgan, Daykundi, Paktika, and Kapisa report low P4P performance primarily due to staffing shortages, limited outreach capacity, and weak defaulter tracking mechanisms. Maternal and child health services are underutilized, and HMIS verification suffers from incomplete registration, missing MCH handbooks, irregular committee meetings, and high staff turnover. Structural gaps—such as limited delivery room equipment, inconsistent tracer and essential drug availability, and substandard hygiene—further compromise service quality. Content-of-care quality remains weak due to underutilization of IMNCI tools, clinical vignettes, and supervision logs, limiting staff competency reinforcement.

Kunduz, Laghman, Logar, and Paktya exhibit significant gaps in family planning, immunization, and maternal health indicators. Weak internal referral systems, inadequate defaulter follow-up, and limited staff knowledge hinder P4P performance. Structural deficits, including missing delivery and maternity equipment, insufficient laboratory and diagnostic capacity, and poor hygiene practices, compromise patient safety and service effectiveness. COVID-19 preparedness remains inconsistent, with limited availability of masks, hand sanitizers, and educational materials.

Zabul, Nooristan, and Farah face pronounced hygiene and sanitation challenges, exacerbated in facilities housed in rental buildings where standards are difficult to enforce. Essential and tracer drug management is weak, with irregular supply, delayed deliveries, and incorrect MAC calculations. Content-of-care quality across ANC, PNC, TB, FP, GMP, and IYCF is consistently suboptimal, reflecting limited staff capacity, weak supervision, and insufficient application of clinical tools. These structural and service quality gaps contribute to poor maternal and child health outcomes and low adherence to Minimum Service Standards (MSS).

Kunar, Panjshir, Kapisa, and Paktya show low content-of-care quality across ANC, PNC, TB, FP, GMP, and IYCF services, primarily due to limited staff knowledge and weak application of clinical protocols. Supervisory tools, including vignettes and logbooks, are underutilized, reducing opportunities for ongoing skill reinforcement. High client volumes, incomplete assessments, and inadequate counseling compromise newborn care, postpartum services, and follow-up for complicated cases. Weak referral linkages exacerbate these gaps, particularly for cesarean section and major surgery cases.

Wardak, Samangan, Sar-e Pul, Badghis, and Badakhshan experience poor outcome quality, with low utilization of maternal and child health services, limited follow-up, and insufficient MSS compliance. Nooristan, Panjshir, and Badghis additionally report low cesarean section and major surgery coverage due to weak referral systems, low outpatient attendance, and shortages of skilled staff, including gynecologists and obstetric surgeons.

Zabul, Nooristan, Farah, Laghman, and Logar are heavily impacted by structural deficiencies, including inadequate hygiene, poor sanitation, limited essential and tracer drug availability, and insufficient facility infrastructure. These deficits reduce service quality and safety, particularly for maternal and neonatal care.

Baghlan, Kunar, Kapisa, Badghis, and Daykundi exhibit systemic weaknesses in HMIS, with poor registration practices, incomplete cross-checking, high staff turnover, and irregular committee meetings. Limited use of supervision vignettes and clinical tools reduces the ability of staff to maintain quality standards and adhere to national guidelines.

Kunar and Panjshir present pockets of effective management, hygiene, and service delivery. Facilities in these provinces demonstrate better organization, adherence to protocols, and functional referral systems. However, systemic gaps—including staffing shortages, weak supervision, limited community engagement, and underutilized clinical tools—persist, limiting overall provincial performance.

Key Performance Gaps and Root Causes:

The field visits identified recurring performance gaps across provinces, driven by both systemic and contextual factors.

Cross-Cutting Systemic Issues

- Weak HMIS practices, including incomplete registration, poor data cross-checking, and the absence of internal verification mechanisms
- Low adherence to national and WHO treatment protocols, leading to poor content-of-care quality
- Inadequate application of supervision tools such as vignettes, IMNCI forms, and clinical logbooks
- Structural deficiencies, including poor hygiene, insufficient equipment, and gaps in essential and tracer drug availability
- Human resource constraints, particularly shortages of skilled female staff and high workload in high-volume facilities

Provincial-Level Variations

- Remote and hard-to-reach provinces experienced lower service utilization due to geographic, cultural, and staffing constraints
- Facilities operating in rented buildings demonstrated weaker compliance with hygiene and MSS requirements
- Provinces with parallel vertical programs faced reporting overlaps and HMIS inconsistencies
- Provinces with stronger leadership and coordination showed comparatively better organization and adherence to standards, despite systemic challenges

These variations highlight the need for province-specific, context-sensitive interventions within a standardized national framework.

Actions Taken During the Field Visits:

During the field visits, the MA team enhanced the capacity of the SP and addressed identified gaps.

Capacity Building

- Delivered training for technical and managerial staff on PIP development using the new structured approach
- Provided refresher guidance on HMIS registration, data accuracy, and verification practices
- Conducted clinical orientation sessions on diagnosis, treatment, and patient counseling for common diseases including pneumonia, diarrhea, TB, Malaria..etc.

Technical Mentoring

- Provided hands-on coaching to the Technical Pharmacy Officers/field supervisors on Essential Drug Management (EDM), tracer drugs, and MAC calculations
- Brief explanation in applying WHO and NSTG treatment protocols
- Facilitated discussions on ethical conduct, patient confidentiality, and respectful care

PIP Technical Leadership

- Technically led the development and revision of provincial PIPs
- Facilitated group work to identify root causes, prioritize actions, and define responsibilities
- Reviewed draft PIPs and provided real-time feedback to ensure feasibility, alignment with data, and integration into routine provincial planning

Recommendations:

The field visit highlights persistent challenges in P4P indicators, HMIS verification, structural quality, content-of-care, and outcome quality across Afghanistan. To address these gaps and improve health service delivery, the following recommendations or The main recommendations are strengthening referral systems, improving community awareness, expanding female workforce capacity, standardizing supervision tools, reinforcing HMIS practices, and addressing structural gaps, which are critical to enhancing health outcomes and achieving improved health services, reduce morbidity and mortality rates across Afghanistan, are proposed, targeting both provincial-level priorities and cross-cutting systemic interventions.

I. Provincial-Level Recommendations

1. Proper usage of Treatment Protocol by Medical Doctors and Midwives in all Health Facilities:

- Implement WHO Treatment protocols for usual diseases including Pneumonia, Diarrhea, TB, and Malaria.
- Implement NSTG and vignettes
- Ensure the vignettes are installed for all Medical Doctors and Midwives working in HFs.

2. Strengthen HMIS Verification and Data Quality

- Establish internal HMIS verification committees in all facilities to improve registration, record-keeping, and data cross-checks.
- Train staff on proper use of registers, coding, and MCH handbooks.
- Pilot mobile-friendly data entry and feedback systems to reduce errors and enhance data visibility.

3. Improve Community Awareness and Engagement

- Strengthen health education through CHWs, CHSs, and community sessions on TT2+, ANC, GMP, Penta-3, and FP.
- Conduct SBCC campaigns to address misconceptions and promote adherence to routine care.
- Institutionalize client feedback mechanisms, including exit interviews, to improve service responsiveness.

4. Institutionalize Structured Technical Mentorship

- Recruit additional Medwive and Doctors to replace the one who are on leave, and avoide missing medwives or Doctors in HFs and meet the P4P target.
- Embed qualified health mentors or district-level roving supervisors to support frontline staff on AMC/EDM, IMCI, surgical documentation, and emergency response protocols.
- Prioritize provinces like Nooristan, Paktika, Farah, Badghis, Zabul, and Uruzgan where training fatigue and reactive learning cycles are prevalent.

5. Strengthen Referral Systems and Service Utilization

- Standardize internal and external referral pathways in all health facilities.
- Train health facility staff and CHWs on referral protocols and defaulter tracking.
- Conduct community awareness campaigns on ANC, PNC, TT2+, institutional delivery, cesarean section, and GMP.
- Deploy female health workers and culturally sensitive strategies to improve access for women.

6. Enhance P4P Performance and Clinical Quality

- Provide on-the-job training for doctors, midwives, and nurses on patient assessment, diagnosis, treatment, and counseling.
- Implement WHO treatment protocols for pneumonia, diarrhea, TB, ANC, PNC, and FP, as well as NSTG for all relevant services.
- Reinforce vignette-based clinical supervision for ANC, PNC, GMP, diarrhea, TB, FP, and PPH.
- Ensure availability of essential equipment, drugs, and maternity kits across all facilities.

7. Improve Health Facility Structural Quality

- Upgrade hygiene and infection prevention, including COVID-19 measures, functional waste pits, sterilization, and PPE availability.
- Strengthen essential drug management (EDM) and tracer drug supply through pharmacist training on MAC calculation and expired drug protocols.
- Ensure functional maternity, FP, and delivery equipment to meet Minimum Service Standards (MSS).

II. Cross-Cutting System Recommendations

1. Enhance PIP Development and Implementation

UNICEF should recommend the extension of the MA project with technical leadership to the MgtWell/HPRO as they have enough experience in MA Project since 2023, to guide PIP development using a new structured approach. MA experts will provide guidance at least until the end of 2026 to ensure proper implementation of province-specific plans.

2. Strengthen TPM and Data-Driven Planning

Regular data collection by TPM on a quarterly basis to provide data for analysis to the MA project, so the MA team can analyze the data, identify gaps, and include them in the PIP for improving health services in all HFs related to the HER2 project.

3. Institutionalize HMIS Verification

All SPs should establish HMIS internal verification committees in health facilities to improve registration practices, cross-check data, and ensure accurate and consistent HMIS reporting.

4. Enhance Content of Care Quality

On-the-job technical training should be conducted for doctors, midwives, and other health staff on history taking, physical examination, lab investigations, diagnosis, treatment, and patient consultation for usual diseases, including pneumonia, diarrhea, TB, and postpartum hemorrhage. Clinical protocols, vignettes, and minimum service standards should be consistently applied to improve the quality of care.

5. Strengthen Ethics and Staff Engagement

Health worker care programs, recognition policies, and structured client-feedback documentation practices should be institutionalized to improve staff motivation, accountability, and patient-centered service delivery.

6. Strengthen Supervisory Capacity

Provincial supervisor roles should be enhanced through mentorship, technical training, and field exposure to support frontline staff and improve adherence to clinical protocols and service quality standards.

7. Improve Provincial Data Systems

Mobile-friendly data capture and feedback tools should be piloted by SPs at provincial level in all provinces to reduce errors, improve data visibility, and enable real-time monitoring of health services.

8. Align and Strengthen Health Policies

The MA project should review the health policies of all SPs, consolidate, and harmonize them with national public health standards and HER objectives, ensuring sequential alignment to guide consistent implementation and strengthen health system governance.

The Recommendations Based on Health Facility-Level Findings will be provided in the next time.

The MA team will conduct field trip after the extension of the contract, as the current contract of the MA project expires till end of Jan 2026, and visit Health Facilities and provide their recommendations based on HF level findings the same as they did in 2024. The above recommendations are intended to serve as a blueprint not only for tactical correction but for structural transformation—ensuring that HER's next iterations are grounded in responsiveness, staff empowerment, and patient dignity.

Conclusion:

The field visits conducted by the MgtWell and HPRO consortium during rounds two and three of 2025 were pivotal for the development of the Performance Improvement Plans (PIPs) using the new structured approach. The MA team presented Provincial Data Packs, identified critical performance gaps, explained the five-phase PIP methodology, and provided technical guidance to support the development of PIPs across 20 provinces.

The assessment revealed that most Service Providers (SPs) had not met the required thresholds in HMIS verification, particularly regarding data accuracy. QQM scores highlighted areas requiring urgent improvement, including hygiene, Essential Drug Management (EDM), tracer drug availability, and clinical care quality—especially the rational use of antibiotics and adherence to diagnosis and treatment protocols. These findings were incorporated into the PIPs with clear recommendations for action, including infrastructure improvements in facilities housed in rental properties where hygienic standards could not be fully maintained.

While disparities persist due to documentation gaps, inconsistent training, and limited supervisory capacity, the system's strength lies in its committed workforce, pockets of excellence, and the willingness of SPs to engage and improve. The MA team's approach—anchored in real-time coaching, contextual adaptation, and collaborative mentorship—translated monitoring into actionable guidance and capacity building.

Sustainable success will depend on embedding these practices into routine provincial operations, scaling localized innovations, and harmonizing national standards with field realities. HER 2 and HER 3 must not only strengthen systems but also cultivate a culture of professional excellence, patient-centered care, and accountability. This report serves not only as a record of findings but as a strategic blueprint to guide transformation, improve service quality, and enhance health outcomes across Afghanistan.

Annexes:

- Annex 1: Provincial Data Packs 20 Provinces
- Annex 2: PIPs of 20 Provinces

Photos:

unicef

Two days Eye Care workshop for HFs staff of Paktia province

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